IN THE PILL BOX

New drugs and their efficacy in reducing the impact of the virus



BARICITINIB

Developed by: Eli Lilly and Incyte Corporation

Type: Janus kinase (JAK) inhibitors

Efficacy: Reduces risk of death by nearly 50 per cent

Status: WHO recommended, FDA approved, not yet approved in India for Covid (already used to treat arthritis and available in the market)

SOTROVIMAB

Developed by: GlaxoSmithKline

Type: Monoclonal antibodies

Efficacy: Risk of hospitalisation and death reduced by 85 per cent

Status: WHO recommended, FDA approved, waiting approval in India

MERCK PILL

Developed by: Ridgeback Biotherapeutics and Merck

Type: Antiviral

Efficacy: Reduces death and risk of hospitalisation by 30-50 per cent

Status: EUA in India but not recommended for use due to safety concerns

PAXLOVID

Developed by: Pfizer

Type: Antiviral

Efficacy: Reduces the risk of hospitalisation and death by 88 per cent when given to unvaccinated people at high risk of severe Covid within five days of the onset of symptoms

Status: FDA approved, applied for EUA in India

REGEN-COV

Developed by: Regeneron

Type: Monoclonal antibodies

Efficacy: Reduced the risk of symptomatic infection by 81 per cent in Delta, not effective against Omicron

Status: Lost FDA approval for use; approved and recommended in India

ENSOVIBEP

Developed by: Novartis

Type: Antiviral

Efficacy: In a study on 407 volunteers, researchers found that the drug cut the risk of hospitalisation by 78 per cent

Status: Applied for FDA EUA, not yet approved in India

BAMLANIVIMAB AND ETESEVIMAB

Developed by: Eli Lilly

Type: Monoclonal antibodies

Efficacy: Showed a 70 per cent reduction in hospitalisations and deaths; not as effective against Omicron

Status: Approved and recommended in India

75 per cent of adult Indians are now fully vaccinated against Covid, and many experts believe this previously acquired immunity explains the milder run of the Omicron wave. And moving forward, it is the world's unvaccinated people, WHO director general Tedros Adhanom Ghebreyesus warned on January 6, who could host a new variant of the virus. There is no foretelling the severity of the disease it could cause. It could be weak like Omicron, infecting only the upper respiratory tract, or severe and unpredictable like Delta, which resulted in acute lung infection. So, while acknowledging that the present surge has settled, Dr V.K. Paul, member, NITI Aayog, warns that we cannot lower our guard. "We have to be ready for any future variants or surges, and continue to be Covid vigilant," he says.

What Covid has also taught us is that there is no such thing as being too careful, as Hong Kong is now discovering. The city of 7.5 million observed the world's most stringent lockdowns, registering only 26,000 cases and some 200 deaths since the start of the pandemic. Pune, also a city of around 7.1 million, has had nearly 1.2 million cases and 20,000 deaths in that time. However, the moment Hong Kong eased its Covid curbs, Omicron spread like wildfire despite 70 per cent of its population being vaccinated. The city recorded its highest ever number of daily cases—4,285—on February 23, and experts predict the figure could soon touch 28,000. Its hospital system is overwhelmed, with a waiting list of around 10,000. The surge is being attributed to the city having very little previous experience in handling Covid patients, especially in such numbers. Hong Kong residents have also had far less exposure to the virus compared to other cities in the world where lockdowns were lifted a year and a half ago.

Living with Covid, thus, will require more than lockdowns. Strain surveillance, strong clinical treatment and rapid vaccination will have to continue being part of any long-term game plan to beat the virus.



A HEALTHY SIGN Empty beds at the Covid care centre in the Commonwealth Games Village in Delhi

KAMAL SINGH/PTI